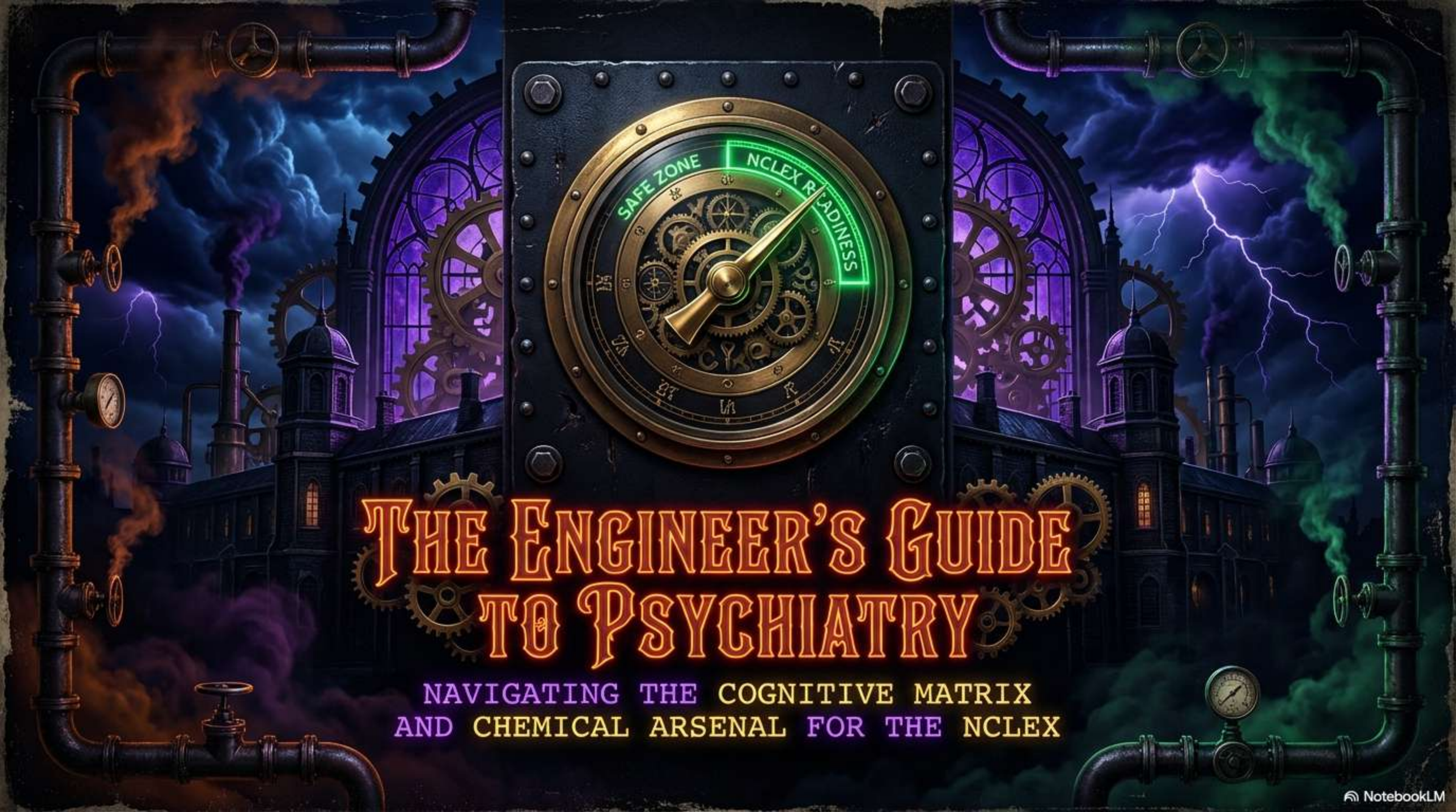
SAFE ZONE
NCLEX READINESS
THE ENGINEER'S GUIDE TO PSYCHIATRY
NAVIGATING THE COGNITIVE MATRIX AND CHEMICAL ARSENAL FOR THE NCLEX
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THE MASTER VALVE: IS THE PATIENT PSYCHOTIC?

The absolute first diagnostic question in any psychiatric scenario.
It determines the entire care plan, treatment, and legality.

THE NON-PSYCHOTIC MIND

- Has insight.
- Is reality-based.
- They know they are sick and know it is affecting their life.

THE PSYCHOTIC MIND

- Has NO insight.
- Is NOT reality-based.
- They do not think they are sick; they believe everyone else has the problem.

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STABILIZING THE NON-PSYCHOTIC PATIENT

THE MECHANISM

The patient maintains insight and reality.

THE PROTOCOL

Treat them exactly like a Med/Surg patient. Use standard, good therapeutic communication.

ENGINEERING COMMANDS

- That must be very overwhelming for you.
- How are you feeling?
- Tell me more about your current feeling.

FOCUS ON

- Reflection, clarification, amplification, and restatement.

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Sensory Glitches in the Psychotic Engine

DELUSIONS

A false, fixed belief or thought.
There is no sensory component.
It is entirely in the head.
(e.g., Paranoid, Grandiose, Somatic)

HALLUCINATIONS

A false, fixed sensory experience
without a referent in reality.
Nothing is actually there.
(Auditory is #1, followed by Visual,
Tactile, Gustatory, Olfactory)

ILLUSIONS

A misinterpretation of sensory
reality with a referent in reality.
There is actually something there,
but the mind misinterprets it.
(e.g., A fire extinguisher is
perceived as a bomb)

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ENGINE FAILURE 1: FUNCTIONAL PSYCHOSIS

THE PATHOLOGY

Chemical imbalance in the brain. They can function in everyday life. Includes Schizo, Schizo, Major, Manics (Schizophrenia, Schizoaffective, Major Depression, acute Mania).

THE PROGNOSIS

This patient has the potential to learn reality.
There is no structural brain damage.

THE 4-STEP ENGINEERING PROTOCOL

1. Acknowledge the feeling (You seem upset.)
2. Present reality (I know you see that demon, but I don't see it.)
3. Set limits (We are not going to talk about that.)
4. Enforce limits (I see you are too ill to stay reality-based, our conversation is over.)

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ENGINE FAILURE 2: PSYCHOSIS OF DEMENTIA

THE PATHOLOGY

Actual brain destruction and damage. Includes Alzheimer's, stroke, Wernicke's, and Organic Brain Syndrome.

Hallmarks: Memory loss, inability to learn.

THE PROGNOSIS

They absolutely cannot learn reality. Do NOT present reality; it will only frustrate them.

THE 2-STEP REDIRECTION PROTOCOL

1. Acknowledge their feeling (*That sounds exciting/scary.*)

2. Redirect them (Let's sit down and talk about church while we wait.)

NOTE: Do not confuse reality orientation (person, place, time) with presenting reality. Orientation is always appropriate.

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ENGINE FAILURE 3: PSYCHOTIC DELIRIUM

THE PATHOLOGY

Temporary, sudden, dramatic, episodic loss of reality due to a chemical imbalance in the body. Causes include UTI, thyroid storm, adrenal crisis, or drug withdrawal (e.g., Delirium Tremens).

THE PROGNOSIS

Temporary and reversible once the underlying cause is resolved.

THE 2-STEP SAFETY PROTOCOL

1. Acknowledge the feeling (I understand you seem to be scared.)

2. Reassure them of safety and temporariness (You are safe, and your fear will go away when you get better.)

INTERVENTION

Treat the underlying medical cause immediately.

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The Chemical Arsenal: Psychopharmacology Rules

The Universal Law of Psychotropics

ALL psychiatric drugs cause hypotension and weight changes (almost always weight gain).

The Protocol for Toxic Effects

If a patient displays toxic effects, HOLD the the drug immediately and CALL the doctor.

The Protocol for Side Effects

If a patient displays expected side effects, teach the patient to keep taking the drug, inform the doctor, and treat the side effects.

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Phenothiazines (The Big Guns)

The Mechanism

1st Gen / Typical Antipsychotics.

They do not cure, they only reduce symptoms. Zines for the Zany.

Large doses = Major tranquilizers.
Small doses = Antiemetics.

Expected Side Effects

A - Anticholinergic (dry mouth)
B - Blurred vision
C - Constipation
D - Drowsiness
E - EPS (Extrapyramidal Syndrome - Parkinsonian symptoms)
F - Fotosensitivity (skin burns)
aG - aGranulocytosis (low WBC, immunosuppression)

Nursing Action

Teach safety for A-F.

Teach reporting of sore throat/infection for G.

Decanoate (D) means long-acting IM form for non-compliant patients.

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Benzodiazepines (The Minor Tranquilizers)

The Mechanism

Anti-anxiety medications. Always have zep, pam, or lam in the name. ZzZ… Zeps for sleeping and sedating.

Indications

Pre-op anesthesia, muscle relaxer, alcohol withdrawal, seizures, fighting mechanical ventilation.

The Time Limit

They work very quickly, but MUST NOT be taken for more than 2 to 4 weeks (risk of dependency).

Expected Side Effects

A – Anticholinergic (dry mouth)
B – Blurred vision
C – Constipation
D – Drowsiness

Nursing Action

#1 Diagnosis is Risk for Injury. Keep on Valium until the long-term antidepressant kicks in.

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MAOIs: The Volatile Elixirs

The Mechanism

Potent antidepressants. Names usually rhyme: Marplan, Nardil, Parnate (PaNaMa).

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The Critical Danger

Hypertensive Crisis.

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The Dietary Protocol

To prevent fatal hypertensive crisis, the patient MUST strictly avoid all foods containing Tyramine.

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The Salad BAR & Forbidden Foods

- Bananas, Avocados, Raisins (any dried fruit).
- NO organ meats, preserved/smoked/cured meats, or hot dogs.
- NO dairy (except mozzarella and cottage cheese). No yogurt.
- NO alcohol, elixirs, tinctures, caffeine, chocolate, or soy sauce.

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Intervention

Teach patients NOT to take OTC medications while on MAOIs.

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Lithium: The Elemental Stabilizer

The Mechanism

Treats Bipolar Disorder. Decreases mania. It stabilizes nerve cell membranes and acts like an electrolyte (closely linked to Sodium).

Toxic Effects

Tremors, metallic taste, severe diarrhea.

HOLD drug and CALL doctor.

Expected Side Effects (The 3 P's)

- Peeing (Polyuria)
- Pooping (Diarrhea)
- Paresthesia (Numbness & tingling - the earliest sign of electrolyte imbalance)

Engineering Protocol

Increase fluids.
Monitor sodium strictly.
Low sodium = Lithium Toxicity.
High sodium = Lithium is Ineffective.
Do not give plain water if sweating heavily; give electrolyte drinks.

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Haldol & The Overheating Engine (NMS)

The Lethal Malfunction - NMS

The Mechanism

Typical 1st Gen Antipsychotic. Basically the same as Thorazine.

The only major antipsychotic tranquilizer that can be given to pregnant women.

The Lethal Malfunction - NMS

Neuroleptic Malignant Syndrome.
A life-threatening neurological disorder causing extreme hyperpyrexia (fever 104F - 108F), anxiety, and tremors.

Differential Diagnosis

Both NMS and EPS cause tremors and anxiety.

The absolute deciding factor is TEMPERATURE.

If fever is present, it is an NMS emergency. Call Rapid Response.

Side Effects

Identical to Phenothiazines (ABCDEFG).

Dosing Rule

Elderly patients must receive half the usual adult dose to prevent NMS.

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Zoloft (Sertraline): The SSRI Interactions

The Mechanism

SSRI Antidepressant.
Mood elevator.

Dosing Protocol

Causes severe insomnia.
Give before noon.
NEVER give at bedtime.

Adolescent Warning

Watch for increased suicidal risk when changing doses in young adults and adolescents.

Dangerous Interactions

Not metabolized in the liver like other drugs, leading to toxic build-ups:

+ St. John's Wort = Serotonin Syndrome (SAD Head: Sweating, Apprehensive/doom, Dizziness, Headache).
+ Warfarin = Bleeding. You must anticipate lowering the dose of Zoloft if taken concurrently.

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The Master Engineer's Blueprint

Assess the Engine First

Is the patient psychotic?
If no, use therapeutic communication.
If yes, locate the failure
(Functional, Dementia, Delirium).

Apply the Exact Protocol

Present reality (Functional),
Redirect (Dementia), or
Reassure Safety (Delirium).

Deploy Chemicals Carefully

Know your side effects (ABCDEFG).
Know your limits (Benzos max 4 weeks).
Know your diet (MAOIs & Tyramine).

Watch the Temperature Gauge

EPS is a side effect.
Fever + Tremors = NMS.
Hold the drug, save the life.

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